

Decision Making Algorithm for INITIAL (new or >72 hrs since prior) Blood Cultures (BC)

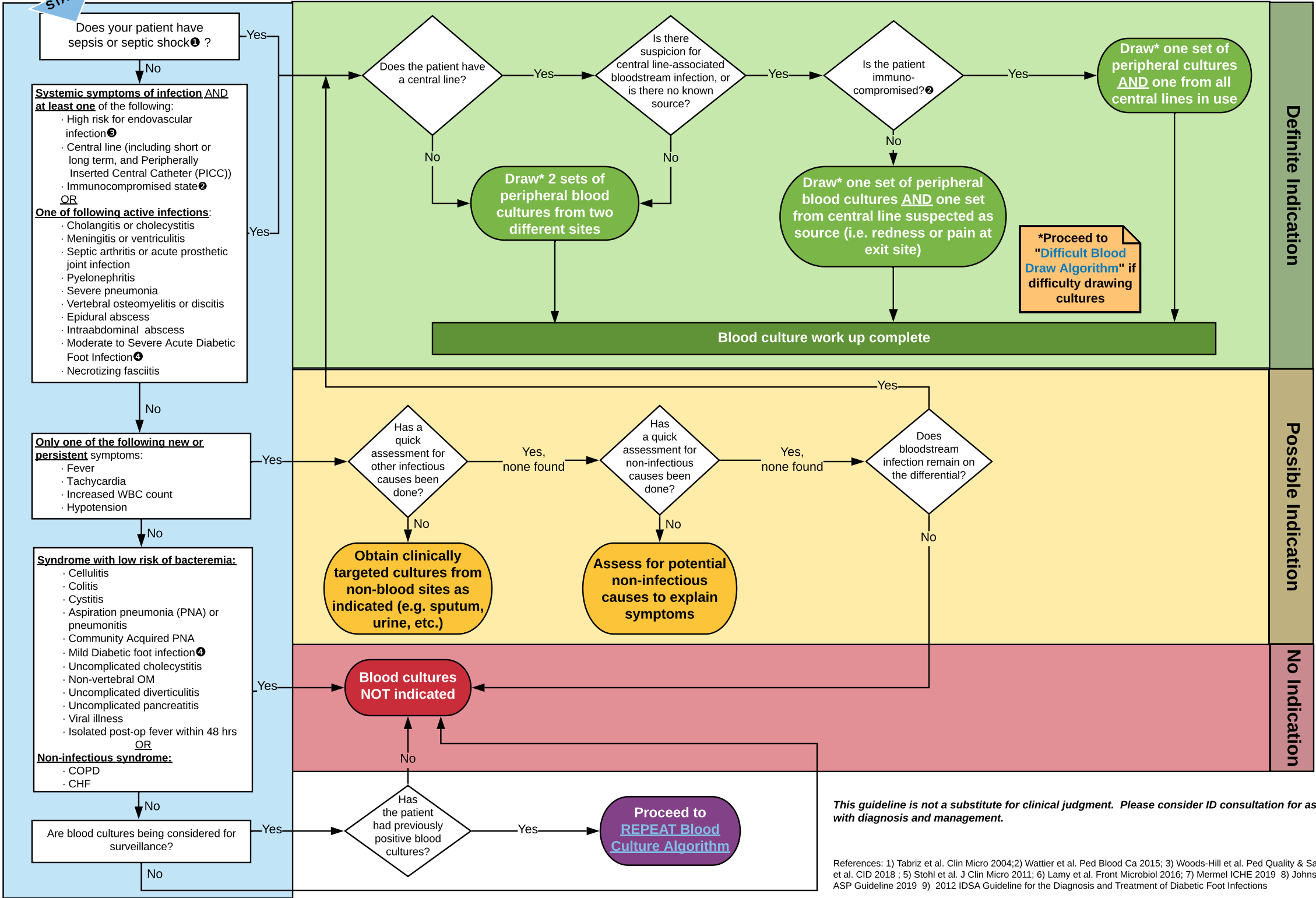
FOOTNOTES:

❶ Sepsis: At least one of the following [T>101 or <96.8 deg F, HR>90, RR>20, WBC>12 or <4, >10% bands] AND Organ dysfunction; Septic shock: Sepsis criteria AND initial lactate >4 or persistent hypotension despite fluid resuscitation

❷ Immunocompromised defined as one or more of the following:
· Active hematologic malignancy
· Malignancy requiring chemotherapy in the last 3 months
· Prior stem cell transplant
· Solid organ transplant on immunosuppressive medication
· Chronic (>14 days) high dose steroids (prednisone>20mg/day or equivalent in last 3 months)
· Immunomodulator or biologic immunosuppressive therapy
· HIV/AIDS (CD4 <200)
· Asplenia
· Neutropenia
· Receipt of other immuno-suppressive medications in the last 12 months

❸ High-risk endovascular infection = Infective endocarditis (IE), septic thrombophlebitis, ICD/pacemaker in place, vascular graft, ventricular assist device (VAD), prosthetic valve or prosthetic material for valve repair, history of IE, unrepaired congenital heart disease

❹ Diabetic foot infection:
· Mild= local infection involving only skin and subcut tissue with erythema 0.5-2cm around ulcer;
· Moderate = local infection with erythema >2cm and no systemic symptoms
· Severe = local above described infection with systemic symptoms or metabolic perturbations



This guideline is not a substitute for clinical judgment. Please consider ID consultation for assistance with diagnosis and management.

References: 1) Tabriz et al. Clin Micro 2004;2) Wattier et al. Ped Blood Ca 2015; 3) Woods-Hill et al. Ped Quality & Safety 2017;4) Miller et al. CID 2018 ; 5) Stohl et al. J Clin Micro 2011; 6) Lamy et al. Front Microbiol 2016; 7) Mermel ICHE 2019 8) Johns Hopkins Hospital ASP Guideline 2019 9) 2012 IDSA Guideline for the Diagnosis and Treatment of Diabetic Foot Infections